



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : دعاء عبدالحليم احمد عبدالحليم

العنوان : سندوة

الحي / المدينة : الخانكة

المحافظة : القليوبية

تاريخ تسجيل الإستمارة : 04-10-2026

رقم البطاقة : 28811241402502 رقم التليفون : 01010625120

العمر (تاريخ الميلاد) : 24-11-1988 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي : _____

المقرر بما فيه : _____

التوقيع: (_____)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي زولاديكس تحت الجلد لمدة 6 شهور

قسم العلاج :
الأورام

[14163]

التعليق على التشخيص : _____

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني

الاسم : د/ندى حسن

الاسم : د/نرمين رمضان

الوظيفة : _____

الوظيفة : _____

الوظيفة : _____

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة رئيس اللجنة ومدير المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / _____ / د. / _____ / د. / _____

قرار اللجنة

التشخيص النهائي : _____

خطة العلاج : _____

المستشفى : _____

قسم : _____

جهة العلاج : _____

نفقات : _____

ملاحظات : _____

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: دعاء عبدالحليم احمد عبدالحليم National ID: 28811241402502	
--	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Goserelin (Zoladex) subcutaneous

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جوستاف
روني
9-17مبادرة رئيس الجمهورية
لدعم صحة المرأة المصريةالبنية التحتية
للصحة

09/16/2026

MDT FORM

Name: <u>د. هاء جود الحليم احمد عبدالمحيم</u>	MRN: <u>13291</u>
Age: <u>38 Y</u>	ID: <u>28811241402502</u>
Menopausal Sta: <u>PREMENOPASUAL</u>	Phone: * <u>01031041821</u>
NRS SCORE: <u>0</u>	Marital Status: <u>MARRIED</u>
BMI: <u></u>	Family History: <u>FREE</u>
Gender: <u></u>	Reason Of MDT: <u>NEW</u>
CO Morbidity: <u>FREE</u>	ECOG Performance Status Ⓢ 0 1 2 3 4
Surgical History: * <u>FREE</u>	

Clinical Data:

BX: INVASIVE DUCT CARCINOMA

ER: 7/8 PR: 6/8 HER2: 0 KI67: 60%

History:

Sonomammography:

ACR C
RT UOQ MULTIPLE MALIGNANT MASS LESIONS WITH FEW IPSILATERAL
LYMPH NODES level I and II.

CESM:

rt breast : multifocal pathologically enhancing malignant process from 8- 9
to 10 o'clock with intervening non mass enhancement , the overall
process (8.3x2.5) associated with ipsilateral pathological level I axillary
LNs (BIRADS 6)
LT breast : no pathological mass or non mass (BIRADSs I)

Biomarker/Molecular

Mets Workup:

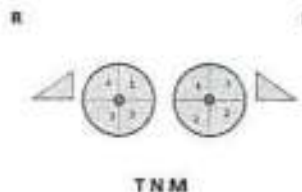
CAP FREE OF METS

Recommendations:

clinically: medium size breast with palpable 2 masses
for possibility of clipping of all lesion then NACT
then BCS
omnia essayy

MDT Outcome & Plan

In view of young age, having N+ve disease, with high Ki-67;
For the insertion of 3 clips in the 3 reported masses.....then portacath
insertion & starting NACT AC x4 followed by Taxotere x4 then re-do
mammosonography and consider Rt WLE & AXLN management accordingly
+ zoladex ovarian function preservation.



Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Clinical Staging:

Pathological Staging:

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د. شيماء لاشين	د. هانيه عيسى		

LU5060615

٨ ٧٧ ١١ ٤ ٤ ١٤ ٠ ٢٥ ٠ ٨

مركز الخدمة - القنصلية
مستوى

عبد الحليم احمد عبد الحليم
نساء



مملكة البحرين
السلطنة

Name: دعاء عبدالحليم احمد عبدالحليم			
MR. : 13291			
Detailed Medical Report			
			السيد
تحية طيبة وبعد,,			
		إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
		رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالتها			
تاريخ وساعة الدخول :		تاريخ وساعة الخروج :	
وبعد الكشف والفحوصات والمتابعة وجد انها			
The patient is a 37-year-old premenopausal individual with no reported family history of cancer and no comorbidities. The clinical history notes a medium-size breast with two palpable masses. Core needle biopsy demonstrated invasive duct carcinoma. Immunohistochemistry showed estrogen receptor 7/8, progesterone receptor 6/8, HER2 0, and Ki-67 60 %. Imaging revealed an ACR C sonomammographic pattern with multiple malignant-appearing right-upper-outer quadrant masses and a few ipsilateral level I and II lymph nodes. Contrast-enhanced spectral mammography demonstrated a multifocal, pathologically enhancing malignant process in the right breast extending from the 8-9 to 10-o'clock position, measuring 8.3 × 2.5 cm with intervening non-mass enhancement and associated ipsilateral pathological level I axillary lymph nodes (BIRADS 6); the left breast was negative (BIRADS I). Metastatic workup was negative for distant disease. The referring team recommended clipping of all lesions, followed by neoadjuvant chemotherapy (NACT) and breast-conserving surgery (BCS). The MDT concluded that, given the patient's young age, node-positive disease, and high Ki-67, a revision of pathology is required to resolve Ki-67 and grade discrepancy. The plan includes insertion of three clips into the three reported masses, placement of a port-a-cath, initiation of NACT with AC × 4 cycles followed by Taxotere × 4 cycles, repeat mammosonography, and consideration of right-sided wide local excision with axillary lymph node management, together with zoladex for ovarian function preservation.			
تحريراً في ,			
		التاريخ : 23-09-2026	الوقت :
		الطبيب المعالج :	
يعتمد ,			



١١

٢٠٢٢/٠١/٢١ تاريخ انتهاء صلاحية البطاقة

يوسف محمد عيسى علي

الجنسية

مصرية

الجنس

رقم البطاقة

٢٠٢٢/٠١/٢١ ٢٨٨١١٤٤١٠٢٠٢



Patient ID: 15753	Patient Name English: Dosa abdhalam
Age	2026/07/09 11:56:13 AM :Report Date

Women's Health Outreach Program

Bilateral Full field Digital Mammography and high-resolution ultrasound.

Clinical summary: 38 years old female patient with right breast lump of 3 months duration .

Right breast:

- **Heterogenously dense mammary parenchyma . (ACR density C)**.
- The right breast shows UOQ posterior 1/3 speculated dense mass lesion with obscured margins located at right axillary tail seen about 10.5 cm from the nipple. Aswell as retroareolar asymmetry see in Upper central location .
- **Ultrasound** revealed large ill defined hypoechoic lobulated mass lesions at 9 :00 position 8 cm away from nipple measures about 1.8 x 1.5 x 1 cmtall more than wide cm at its maximum diameters (index mass lesion) , another assymetrical hypoechoic illdefined lesion located at 12 ° clock position measuring 2.2 x 2.4 cm mounting to the retroareolar asymmetry seen in mammographic films .
- Normal nipple/areolar complex.
- Few enlarged axillary lymph nodes at level I, II with central hila but thickened cortices largest one measures about 11 x 4.9 mm , likely pathological .

Left breast:

- **Heterogenously Scattered fibro glandular densities (ACR density C)**.
- No masses, asymmetric densities, distortion or suspicious clusters of micro calcifications.
- **Ultrasound** demonstrated prominent glandular elements of the breast tissue with mottled appearance.
- Multiple small dispersed subcentimetric thin-walled anechoic cysts.
- Few prominent retroareolar ducts .
- Normal skin/subcutaneous tissue and nipple/areolar complex.
- Non-specific axillary LNs.

Impression:

- **Right breast (BIRADS 4):** Right UOQ multiple malignant mass lesions with few ipsilateral pathological axillary lymph nodes as described. For biopsy and histopathological correlation .

CESM OR DYMANIC BREAST mri IS RECOMMENDED .

- **Left breast (BIRADS 2):** left breast adenotic changes & prominent ducts .

Much obliged,

Dr. Sarah Hussein , MD



اسم المريض بالإنجليزية: Doaa Abd Alhaleem Ahmed Abd Alhalee	كود المريض: 99172
تاريخ التقرير: 2026/09/09 09:41:32 AM	الدين: TV، سنة: 9 شهر

Contrast enhanced Spectral Mammography

CLINICAL SUMMARY:

- Right breast malignant mass pathologically proved to be invasive ductal carcinoma, grade II, for pre-management evaluation.

TECHNIQUE:

- CC & MLO low energy & recombined (post contrast) images

FINDINGS:

Background Uptake: mild background enhancement

ACR: c (heterogeneously dense breast parenchyma).

RIGHT BREAST:

- **Low energy (Non contrast) images:** Mid outer, zone B, obscured mass is noted.
- **Recombined (Post contrast) images:**
 - Corresponding mid-outer (8-9 o'clock) heterogeneously enhancing mass of marked conspicuity is noted measuring (3.3 x 2.8 x 2.6) cm along its maximum dimensions, being at about 4.5 cm from the nipple and 0.2 cm from the skin (as detected by complementary US examination).
 - Another UOQ (10 o'clock) heterogeneously enhancing mass is seen measuring about (2.2 x 1.8 x 2) cm along its maximum dimensions, being postero-superior to the index mass by about 3 cm.
 - Intervening non-mass enhancement of mild conspicuity is noted with the overall process measuring about (8.3 x 2.5) cm, being at about 4.5 cm from the nipple.
- Normal skin / nipple / areolar complex.
- By **complementary US:** Few ipsilateral pathologically enlarged level I axillary lymph nodes are noted.

LEFT BREAST:

- **Low energy (Non contrast) images** No masses, asymmetry or distortion or suspicious microcalcifications.
- **Recombined (Post contrast) images** show, no pathological uptake.
- Normal skin / nipple / areolar complex.

IMPRESSION:

RIGHT BREAST: multifocal pathologically enhancing malignant process (from 8-9 to 10 o'clock) associated with ipsilateral pathologically enlarged level I axillary lymph nodes (**BIRADS 6**).

LEFT BREAST: No pathological mass or non-mass enhancement (**BIRADS 1**)

Best regards,

Dr. Sherihan Fakhry, MD





MINISTRY

قسم الباثولوجي

الهيئة العامة للمستشفيات والمعاهد التعليمية
المعهد القومي للكبد والجهاز الهضمي
والأمراض المعدية

PATHOLOGY DEPARTMENT HISTOPATHOLOGICAL REPORT

Patient name: دعاء عبد الحليم احمد عبد الحليم Age: 38 years Sex: female
Request type: ميانزه Sub request type:
Hospital no: Pathology no.: 6388-2026
Department: صحة مرآه Date of result:
Referred by: د/ايمن عادل Date of biopsy: 14/7/2026
Nature of specimen: tru-cut biopsy I.D. Number: 28811241402502

Clinical diagnosis: Right breast mass

Gross pathology:

Multiple grayish white tissue cores up to 1cm long, totally submitted.

Immunohistochemical Technique:

Sections were prepared from the paraffin block, processed in Benchmark Ultra machine (Ventana) and stained by rabbit monoclonal antibodies using Ultra view HRP DAB as chromogen and hematoxylin as counter-stain. The sections were treated against: ER- PR- Her2- Ki67

Microscopic picture: Section examined revealed core of tumor showing invasive duct carcinoma of no special type (NST). The tumor forms nests, tubule and cribriform pattern infiltrating among desmoplastic stroma. Tumor cells showing moderate anaplasia and moderate tendency to form tubules. There is moderate number of mitotic figures. No carcinoma insitu component is seen.

Immunohistochemical Result:

ER: Moderate nuclear staining in 90% of tumor cells (Allred Score 7/8).

PR: Moderate nuclear staining of 40% of tumor cells (Allred Score 6/8).

Her2: No membranous staining of tumor cells (score 0). Negative

Ki67: Positive nuclear staining of 60% of tumor cells.

Diagnosis: Left breast mass, tru cut biopsy:

Invasive Duct Carcinoma, (NST) Grade II.

Signature:

Dr. Mona Mohamed.M.D

3

Pathology Lab. Manager

EASMA REFAAT M.D

Patient's Name: Doaa Abdelhalim Ahmed Abdelhalim
Date: 2026-08-24
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

Examination:

■ **Computed tomography of the chest, abdomen and pelvis**

Technical data: 5 mm CT scans of the abdomen and pelvis with IV contrast

Clinical data: diagnosed right breast cancer

Comparison: none

Findings:

• **Chest findings:**

- Right breast soft tissue density lesions with skin thickening
- Clear both lungs fields with no active parenchymatous infiltrates.
- No hilar or mediastinal masses or sizable LNs.
- Normal appearance of the tracheobronchial tree.
- No evidence of pleural effusion or thickening.
- Average cardiac size, no pericardial effusion.

• **Abdomen and pelvis findings:**

- The liver shows average size with beaver tail variant , smooth contours, homogeneous hepatic parenchyma density and no evidence of focal lesions.
- No intra hepatic biliary dilatation.
- Normal caliber and homogenous enhancement of the portal vein.
- Normal CT appearance of the spleen, gall bladder, pancreas, kidneys and adrenal glands
- The urinary bladder shows average distention, no wall thickening, no stones, diverticulae or masses.
- No suspicious abdominal or pelvis enlarged LNs
- No CT evidence of intestinal obstruction
- No pneumoperitoneum
- No free or localized abdominal or pelvis fluid collection
- No suspicious bony lesions

Opinion:

- **Right breast lesions for dedicated breast studies correlation**
- **No definite pulmonary nodules or masses**
- **No suspicious abdominal or pelvis metastatic lesions on CT base**
- **No suspicious bony lesions**

Recommendation:

- **Follow up**

Regards, Dr. Ahmed Rezk, MD.

Ahmed

Ahmed

4

Patient Name	دعاء عبداللطيف احمد عبداللطيف	Case No.	360306
Sex/Age	Female 37 Y	Patient No.	26306
Registered Date	24/8/2026 03:41:24 PM	Referred By Dr.	Interventional Radiology
Validation Date	24/9/2026 11:02:40 AM	Branch	Gustave Roussy
Reporting Date	24/9/2026 12:43:52 PM	Pathology No.	2731/26

Pathology Report

Clinical Data: Right breast mass (BIRADS 4).

Nature of Specimen: Tru-cut biopsy, collected on 24/08/2026, at 10:30 am. Cold ischemia time is 20 seconds.

Gross Examination:

- Four grey, white tissue cores, ranging in size between 0.5 cm & 2 cm long, totally submitted.

Technique:

- Sections prepared from paraffin block for immune-staining using Benchmark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination:

-Histologic type: Invasive duct carcinoma.
-Histologic grade: 2 (Differentiation 3, pleomorphism 2, mitosis 1).
- DCIS: Low grade ductal carcinoma in situ (Cribiform with comedo type necrosis).
- Microcalcification: Absent.
- Angiolymphoma: Absent.
- Perineural invasion: Absent.
- Tumor infiltrating lymphocytes (TILs): Mild.

Results:

* IPT (All controls showed appropriate reactivity):
- ER: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 30% of tumor cells (score 6/8).
- Her2: Negative membrane reaction in tumor cells (score 0).
- Ki67%: 30%.

Conclusion:

Right breast mass, Tru-cut biopsy:
- Invasive duct carcinoma, Grade 2.
IPT:
- ER: Positive (score 8/8).





Gustave Roussy International - Egypt Pathology Lab
جوستاف روسي انترناتيونال - مصر - مختبر الأمراض الجلدية



360306	Case No.	دعاء عبدالعليم احمد عبدالعليم
26306	Patient No.	Female 37 Y
Interventional Radiology	Referred By Dr.	24/8/2026 03:41:24 PM
Gustave Roussy	Branch	24/9/2026 11:02:40 AM
2731/26	Pathology No.	24/9/2026 12:43:52 PM
		Reporting Date

- PR: Positive (score 8/8).
- Her2: Negative (score 0).
- Ki67: 30%.

****Scoring system: "ER, PR"**
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.
**** Scoring system: "Her2"**
Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature:
Dr. Walaah Ibrahim Ghonaim, MD.



مصر - مختبر الأمراض الجلدية - جوستاف روسي انترناتيونال - مصر - مختبر الأمراض الجلدية
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mistr El Qadema - Cairo, Egypt

جوستاف
روسي
9-17



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

السنة المصرية
1447
هـ

09/16/2026

MDT FORM

Name: دعاء عبد الحليم احمد عبد الحليم	MRN: 13291
Age: 38 Y	ID: 28811241402502
Menopausal Sta: PREMENOPASUAL	Phone:* 01031041821
NRS SCORE: 0	Marital Status: MARRIED
BMI:	Family History: FREE
Gender:	Reason Of MDT: NEW
CO Morbidity: FREE	ECOG Performance Status © 0 1 2 3 4
Surgical History:* FREE	

Clinical Data:

BX: INVASIVE DUCT CARCINOMA
ER: 7/8 **PR:** 6/8 **HER2:** 0 **KI67:** 60%

History:

Sonomammography:

ACR C
 RT UOQ MULTIPLE MALIGNANT MASS LESIONS WITH FEW IPSILATERAL LYMPH NODES level I and II

CESM:

rt breast : multifocal pathologically enhancing malignant process from 8- 9 to 10 o'clock with intervening non mass enhancement , the overall process (8.3x2.5) associated with ipsilateral pathological level I axillary LNs (BIRADS 6)
 LT breast : no pathological mass or non mass (BIRADSs I)

Biomarker/Molecular

Mets Workup:

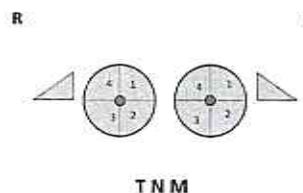
CAP FREE OF METS

Recommendations:

clinically: medium size breast with palpable 2 masses
 for possibility of clipping of all lesion then NACT
 then BCS
 omnia essayy

MDT Outcome & Plan

In view of young age, having N+ve disease, with high Ki-67:
 For the insertion of 3 clips in the 3 reported masses.....then portacath insertion & starting NACT AC x4 followed by Taxotere x4 then re-do mammosonography and consider Rt WLE & AXLN management accordingly + zoladex ovarian function preservation.



Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Clinical Staging:

Pathological Staging:

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د شيماء لاشين	د المنية عيسوي		

Patient ID: 15753	Patient Name English: Doaa abdlhalem
Age	2026/07/09 11:56:13 AM :Report Date

Women's Health Outreach Program

Bilateral Full field Digital Mammography and high-resolution ultrasound.

Clinical summary: 38 years old female patient with right breast lump of 3 months duration .

Right breast:

- **Heterogenously dense mammary parenchyma . (ACR density C)**.
- The right breast shows UOQ posterior 1/3 speculated dense mass lesion with obscured margins located at right axillary tail seen about 10.5 cm from the nipple. Aswell as retroareolar asymmetry see in Upper central location .
- **Ultrasound** revealed large ill defined hypoechoic lobulated mass lesions at 9 :00 position 8 cm away from nipple measures about 1.8 x 1.5 x 1 cmtall more than wide cm at its maximum diameters (index mass lesion) , another assymetrical hypoechoic illdefined lesion located at 12 ° clock position measuring 2.2 x 2.4 cm mounting to the retroareolar asymmetry seen in mammographic films .
- Normal nipple/areolar complex.
- Few enlarged axillary lymph nodes at level I, II with central hila but thickened cortices largest one measures about 11 x 4.9 mm , likely pathological .

Left breast:

- **Heterogenously Scattered fibro glandular densities (ACR density C)**.
- No masses, asymmetric densities, distortion or suspicious clusters of micro calcifications.
- **Ultrasound** demonstrated prominent glandular elements of the breast tissue with mottled appearance.
- Multiple small dispersed subcentimetric thin-walled anechoic cysts.
- Few prominent retroareolar ducts .
- Normal skin/subcutaneous tissue and nipple/areolar complex.
- Non-specific axillary LNs.

Impression:

- **Right breast (BIRADS 4):** Right UOQ multiple malignant mass lesions with few ipsilatera pathological axillary lymph nodes as described. For biopsy and histopathological correlation .

CESM OR DYMANIC BREAST mri IS RECOMMENDED .

- **Left breast (BIRADS 2):** left breast adenotic changes & prominent ducts .

Much obliged,

Dr. Sarah Hussein , MD



اسم المريض بالإنجليزية: Doaa Abd Alhaleem Ahmed Abd Alhalee	كود المريض: 99172
تاريخ التقرير: 2026/09/09 09:41:32 AM	السن: ٣٧ سنة, ٩ شهر

Contrast enhanced Spectral Mammography

CLINICAL SUMMARY:

- Right breast malignant mass pathologically proved to be invasive ductal carcinoma, grade II, for pre-management evaluation.

TECHNIQUE:

- CC & MLO low energy & recombined (post contrast) images

FINDINGS:

Background Uptake: mild background enhancement

ACR: c (heterogeneously dense breast parenchyma).

RIGHT BREAST:

- **Low energy (Non contrast) images:** Mid outer, zone B, obscured mass is noted.
- **Recombined (Post contrast) images:**
 - Corresponding mid-outer (8-9 o'clock) heterogeneously enhancing mass of marked conspicuity is noted measuring (3.3 x 2.8 x 2.6) cm along its maximum dimensions, being at about 4.5 cm from the nipple and 0.2 cm from the skin (as detected by complementary US examination).
 - Another UOQ (10 o'clock) heterogeneously enhancing mass is seen measuring about (2.2 x 1.8 x 2) cm along its maximum dimensions, being postero-superior to the index mass by about 3 cm.
 - Intervening non-mass enhancement of mild conspicuity is noted with the overall process measuring about (8.3 x 2.5) cm, being at about 4.5 cm from the nipple.
- Normal skin / nipple / areolar complex.
- By **complementary US:** Few ipsilateral pathologically enlarged level I axillary lymph nodes are noted.

LEFT BREAST:

- **Low energy (Non contrast) images** No masses, asymmetry or distortion or suspicious microcalcifications.
- **Recombined (Post contrast) images** show, no pathological uptake.
- Normal skin / nipple / areolar complex.

IMPRESSION:

RIGHT BREAST: multifocal pathologically enhancing malignant process (from 8-9 to 10 o'clock) associated with ipsilateral pathologically enlarged level I axillary lymph nodes (**BIRADS 6**).

LEFT BREAST: No pathological mass or non-mass enhancement (**BIRADS 1**)

Best regards,

Dr. Sherihan Fakhry, MD





MINISTRY

قسم الباثولوجي

الهيئة العامة للمستشفيات والمعاهد التعليمية

المعهد القومي للكبد والجهاز الهضمي

والأمراض المعدية

PATHOLOGY DEPARTMENT HISTOPATHOLOGICAL REPORT

Patient name: دعاء عبد الحليم احمد عبد الحليم Age: 38 years Sex: female
Request type: مياثره Sub request type:
Hospital no: Pathology no.: 638B-2026
Department: صحة مرآه Date of result:
Referred by: د/ايمن عادل Date of biopsy: 14/7/2026
Nature of specimen: tru-cut biopsy I.D. Number: 28811241402502

Clinical diagnosis: Right breast mass

Gross pathology:

Multifocal grayish white tissue cores up to 1cm long, totally submitted.

Immunohistochemical Technique:

Sections were prepared from the paraffin block, processed in Benchmark Ultra machine (Ventana) and stained by rabbit monoclonal antibodies using Ultra view HRP DAB as chromogen and hematoxylin as counter-stain. The sections were treated against: ER- PR- Her2- Ki67

Microscopic picture: Section examined revealed core of tumor showing invasive duct carcinoma of no special type (NST). The tumor forms nests, tubule and cribriform pattern infiltrating among desmoplastic stroma. Tumor cells showing moderate anaplasia and moderate tendency to form tubules. There is moderate number of mitotic figures. No carcinoma insitu component is seen.

Immunohistochemical Result:

ER: Moderate nuclear staining in 90% of tumor cells (Allred Score 7/8).

PR: Moderate nuclear staining of 40% of tumor cells (Allred Score 6/8).

Her2: No membranous staining of tumor cells (score 0). Negative

Ki67: Positive nuclear staining of 60% of tumor cells.

Diagnosis: Left breast mass, tru cut biopsy:

Invasive Duct Carcinoma, (NST) Grade II.

Signature:

Dr. Mona Mohamed.M.D

3

Pathology Lab. Manager

RASMA REFAAT M.D

Patient's Name: Doaa Abdelhalim Ahmed Abdelhalim
Date: 2026-08-24
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

Examination:

■ **Computed tomography of the chest, abdomen and pelvis**

Technical data: 5 mm CT scans of the abdomen and pelvis with IV contrast

Clinical data: diagnosed right breast cancer

Comparison: none

Findings:

■ **Chest findings:**

- Right breast soft tissue density lesions with skin thickening
- Clear both lungs fields with no active parenchymatous infiltrates.
- No hilar or mediastinal masses or sizable LNs.
- Normal appearance of the tracheobronchial tree.
- No evidence of pleural effusion or thickening.
- Average cardiac size, no pericardial effusion.

■ **Abdomen and pelvis findings:**

- The liver shows average size with beaver tail variant , smooth contours, homogeneous hepatic parenchyma density and no evidence of focal lesions.
- No intra hepatic biliary dilatation.
- Normal caliber and homogenous enhancement of the portal vein.
- Normal CT appearance of the spleen, gall bladder, pancreas, kidneys and adrenal glands
- The urinary bladder shows average distention, no wall thickening, no stones, diverticulae or masses.
- No suspicious abdominal or pelvis enlarged LNs
- No CT evidence of intestinal obstruction
- No pneumoperitoneum
- No free or localized abdominal or pelvis fluid collection
- No suspicious bony lesions

Opinion:

- **Right breast lesions for dedicated breast studies correlation**
- **No definite pulmonary nodules or masses**
- **No suspicious abdominal or pelvis metastatic lesions on CT base**
- **No suspicious bony lesions**

Recommendation:

- **Follow up**

Regards, Dr. Ahmed Rezk, MD.

Ahmed

Ahmed

4

Patient Name	رعام عبدالحليم احمد عبدالحليم	Case No.	360306
Sex/Age	Female 37 Y	Patient No.	26306
Registered Date	24/8/2026 03:41:24 PM	Referred By Dr.	Interventional Radiology
Validation Date	24/9/2026 11:02:40 AM	Branch	Gustave Roussy
Reporting Date	24/9/2026 12:43:52 PM	Pathology No.	2731/26

12C91

Pathology Report

Clinical Data: Right breast mass (BIRADS 4).

Nature of Specimen: Tru-cut biopsy, collected on 24/08/2026, at 10:30 am. Cold ischemia time is 20 seconds.

Gross Examination:

- Four grey, white tissue cores, ranging in size between 0.5 cm & 2 cm long, totally submitted.

Technique:

- Sections prepared from paraffin block for immune-staining using Benchmark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination:

- Histologic type: Invasive duct carcinoma.
- Histologic grade: 2 (Differentiation 3, pleomorphism 2, mitosis 1).
- DCIS: Low grade ductal carcinoma in situ (Cribiform with comedo type necrosis).
- Micrometastatic: Absent
- Angiolymphatic invasion: Absent
- Perineural invasion: Absent
- Tumor infiltrating lymphocytes (TILs): Mild.

Results:

* IPT (All controls showed appropriate reactivity):

- ER: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 30% of tumor cells (score 6/8).
- Her2: Negative membrane reaction in tumor cells (score 0).
- Ki67%: 30%.

Conclusion:

Right breast mass, Tru-cut biopsy:

- Invasive duct carcinoma, Grade 2.
- IPT:
- ER: Positive (score 8/8).



غرفة رقم (4) - الشعاع الأول - مبنى رقم (987) - كورنيش النيل - مصر الجديدة - طابق 5 - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mizr El Qadema - Cairo, Egypt

Patient Name	رعاء عبدالحليم احمد عبدالحليم	Case No.	360306
Sex/Age	Female 37 Y	Patient No.	26306
Registered Date	24/8/2026 03:41:24 PM	Referred By Dr.	Interventional Radiology
Validation Date	24/9/2026 11:02:40 AM	Branch	Gustave Roussy
Reporting Date	24/9/2026 12:43:52 PM	Pathology No.	2731/26

- PR: Positive (score 8/8).
- Her2: Negative (score 0).
- Ki67%: 30%.

**Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

** Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature:

Dr. Walaa Ibrahim Chonaim, MD.



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر (القاهرة) - القاهرة 5 - مصر
 Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mistr El Qadema - Cairo, Egypt

جوستاف
زوسافي
9-17



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

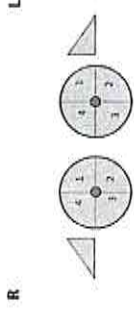
السلطة المصرية
للطب

09/16/2026

MDT FORM

Name:	دعاء متناظم احد عيانتهم	MRN:	13291
Age:	38 Y	ID:	28811241402502
Menopausal Sta	PREMENOPASUAL	Phone: *	01031041821
NRS SCORE:	0	Marital Status:	MARRIED
BMI:		Family History:	FREE
Gender		Reason Of MDT:	NEW
CO Morbidity:	FREE	ECOG Performance Status	0 1 2 3 4
Surgical History:*	FREE		

Clinical Data:



BX: INVASIVE DUCT CARCINOMA
ER: 7/8 **PR:** 6/8 **HER2:** 0 **KI67:** 60%

History:

Sonomammography:
 ACR C
 RT UOQ MULTIPLE MALIGNANT MASS LESIONS WITH FEW IPSILATERAL LYMPH NODES level I and II

CESM:
 rt breast : multifocal pathologically enhancing malignant process from 8-9 to 10 o'clock with intervening non mass enhancement , the overall process (8.3x2.5) associated with ipsilateral pathological level I axillary LN's (BIRADS 6)
 LT breast : no pathological mass or non mass (BIRADSs 1)

Biomarker/Molecular

Mets Workup:
 CAP FREE OF METS

Recommendations:
 clinically: medium size breast with palpable 2 masses for possibility of clipping of all lesion then NACT then BCS

MDT Outcome & Plan
 In view of young age, having N+ve disease, with high Ki-67: For the insertion of 3 clips in the 3 reported masses.....then portacath insertion & starting NACT AC x4 followed by Taxotere x4 then re-do mammosonography and consider Rt WLE & AXLN management accordingly + zoladex ovarian function preservation.

Clinical Staging:

Pathological Staging:

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د. شيماء لاشين	د. لبلبة عيسوي		

جمهورية مصر العربية
بطاقة تحقيق الشخصية

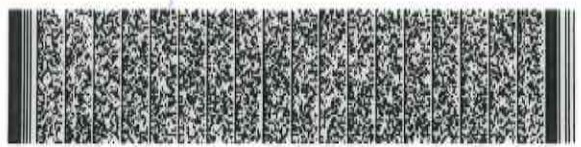


دعام
عبدالحليم احمد عبدالحليم
سندوة
مركز الخانكة - القليوبية

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ربة منزل
انثى مسلمة متزوجة
بسام محمد عوده عابد
البطاقة سارية حتى ٢٠٢٣/٠٦/٢١ ١١



Patient ID: 15753	Patient Name English: Doaa abdihalem
Age	2026/07/09 11:56:13 AM : Report Date

Women's Health Outreach Program

Bilateral Full field Digital Mammography and high-resolution ultrasound.

Clinical summary: 38 years old female patient with right breast lump of 3 months duration .

Right breast:

- **Heterogenously dense mammary parenchyma . (ACR density C).**
- The right breast shows UOQ posterior 1/3 speculated dense mass lesion with obscured margins located at right axillary tail seen about 10.5 cm from the nipple. Aswell as retroareolar asymmetry see in Upper central location .
- **Ultrasound** revealed large ill defined hypoechoic lobulated mass lesions at 9 :00 position 8 cm away from nipple measures about 1.8 x 1.5 x 1 cmtall more than wide cm at its maximum diameters (index mass lesion) , another assymetrical hypoechoic illdefined lesion located at 12 o'clock position measuring 2.2 x 2.4 cm mounting to the retroareolar asymmetry seen in mammographic films .
- Normal nipple/areolar complex.
- Few enlarged axillary lymph nodes at level I, II with central hila but thickened cortices largest one measures about 11 x 4.9 mm , likely pathological .

Left breast:

- **Heterogenously Scattered fibro glandular densities (ACR density C).**
- No masses, asymmetric densities, distortion or suspicious clusters of micro calcifications.
- **Ultrasound** demonstrated prominent glandular elements of the breast tissue with mottled appearance.
- Multiple small dispersed subcentimetric thin-walled anechoic cysts.
- Few prominent retroareolar ducts .
- Normal skin/subcutaneous tissue and nipple/areolar complex.
- Non-specific axillary LNs.

Impression:

- **Right breast (BIRADS 4):** Right UOQ multiple malignant mass lesions with few ipsilatera pathological axillary lymph nodes as described. For biopsy and histopathological correlation .

CESM OR DYMANIC BREAST mri IS RECOMMENDED .

- **Left breast (BIRADS 2):** left breast adenotic changes & prominent ducts .

Much obliged,

Dr. Sarah Hussein , MD

①

Doaa Abd Alhaleem Ahmed Abd Alhalee	اسم المريض بالإنجليزية: 99172	كود المريض:
2026/09/09 09:41:32 AM	تاريخ التقرير: ٩ شهر	السن: ٣٧ سنة

Contrast enhanced Spectral Mammography

CLINICAL SUMMARY:

- Right breast malignant mass pathologically proved to be invasive ductal carcinoma, grade II, for pre-management evaluation.

TECHNIQUE:

- CC & MLO low energy & recombined (post contrast) images

FINDINGS:

Background Uptake: mild background enhancement

ACR: c (heterogeneously dense breast parenchyma).

RIGHT BREAST:

- **Low energy (Non contrast) images:** Mid outer, zone B, obscured mass is noted.
- **Recombined (Post contrast) images:**
 - Corresponding mid-outer (8-9 o'clock) heterogeneously enhancing mass of marked conspicuity is noted measuring (3.3 x 2.8 x 2.6) cm along its maximum dimensions, being at about 4.5 cm from the nipple and 0.2 cm from the skin (as detected by complementary US examination).
 - Another UOQ (10 o'clock) heterogeneously enhancing mass is seen measuring about (2.2 x 1.8 x 2) cm along its maximum dimensions, being postero-superior to the index mass by about 3 cm.
 - Intervening non-mass enhancement of mild conspicuity is noted with the overall process measuring about (8.3 x 2.5) cm, being at about 4.5 cm from the nipple.
 - Normal skin / nipple / areolar complex.
 - By **complementary US:** Few ipsilateral pathologically enlarged level I axillary lymph nodes are noted.

LEFT BREAST:

- **Low energy (Non contrast) images** No masses, asymmetry or distortion or suspicious microcalcifications.
- **Recombined (Post contrast) images** show, no pathological uptake.
- Normal skin / nipple / areolar complex.

IMPRESSION:

RIGHT BREAST: multifocal pathologically enhancing malignant process (from 8-9 to 10 o'clock) associated with ipsilateral pathologically enlarged level I axillary lymph nodes (**BIRADS 6**).

LEFT BREAST: No pathological mass or non-mass enhancement (**BIRADS 1**)

Best regards,

Dr. Sherihan Fakhry, MD

(2)



MINISTRY

قسم الباثولوجيا

الهيئة العامة للمستشفيات والمعاهد التعليمية
(المستشفى) نفوسى للكبد والجهاز الهضمي
والأمراض المعدية

PATHOLOGY DEPARTMENT HISTOPATHOLOGICAL REPORT

Patient name: دعاء عبد الحليم احمد عبد الحليم Age: 38 years Sex: female
Request type: ميائنة Sub request type:
Hospital no: Pathology no.: 638B-2026
Department: صده مرآة Date of result:
Referred by: د. ليمان علي Date of biopsy: 14/7/2026
Nature of specimen: tru-cut biopsy I.D. Number: 28811241402502

Clinical diagnosis: Right breast mass

Request pathology:

Multicystic grayish white tissue cores up to 1cm long, totally submitted.

Immunohistochemical Technique:

Sections were prepared from the paraffin block, processed in Benchmark Ultra machine (Ventana) and stained by rabbit monoclonal antibodies using Ultra view HRP DAB as chromogen and hematoxylin as counter-stain. The sections were treated against ER- PR- Ki67- Ki167

Microscopic picture:

Section examined revealed core of tumor showing invasive duct carcinoma of no special type (NST). The tumor forms nests, tubule and cribriform pattern infiltrating among desmoplastic stroma. Tumor cells showing moderate anaplasia and moderate tendency to form tubules. There is moderate number of mitotic figures. No carcinoma in situ component is seen.

Immunohistochemical Result:

ER: Moderate nuclear staining in 90% of tumor cells (Allred Score 7/8).

PR: Moderate nuclear staining of 40% of tumor cells (Allred Score 6/8).

Her2: No membranous staining of tumor cells (score 0). Negative

Ki67: Positive nuclear staining of 60% of tumor cells.

Diagnosis: Left breast mass, tru cut biopsy:

Invasive Duct Carcinoma, (NST) Grade II.

Signature:

Dr. Mona Mohamed.M.D

Pathology Lab. Manager

RASMA REFAAT M.D

3

Patient's Name: Doaa Abdelhalim Ahmed Abdelhalim
Date: 2026-08-24
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

Examination:

- **Computed tomography of the chest, abdomen and pelvis**

Technical data: 5 mm CT scans of the abdomen and pelvis with IV contrast

Clinical data: diagnosed right breast cancer

Comparison: none

Findings:

- **Chest findings:**
- Right breast soft tissue density lesions with skin thickening
- Clear both lungs fields with no active parenchymatous infiltrates.
- No hilar or mediastinal masses or sizable LNs.
- Normal appearance of the tracheobronchial tree.
- No evidence of pleural effusion or thickening.
- Average cardiac size, no pericardial effusion.
- **Abdomen and pelvis findings:**
- The liver shows average size with beaver tail variant , smooth contours, homogeneous hepatic parenchyma density and no evidence of focal lesions.
- No intra hepatic biliary dilatation.
- Normal caliber and homogenous enhancement of the portal vein.
- Normal CT appearance of the spleen, gall bladder, pancreas, kidneys and adrenal glands
- The urinary bladder shows average distention, no wall thickening, no stones, diverticulae or masses.
- No suspicious abdominal or pelvis enlarged LNs
- No CT evidence of intestinal obstruction
- No pneumoperitoneum
- No free or localized abdominal or pelvis fluid collection
- No suspicious bony lesions

Opinion:

- **Right breast lesions for dedicated breast studies correlation**
- **No definite pulmonary nodules or masses**
- **No suspicious abdominal or pelvis metastatic lesions on CT base**
- **No suspicious bony lesions**

Recommendation:

- **Follow up**

Ahmed

Regards, Dr. Ahmed Rezk, MD.

Ahmed

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